



Leicestershire Partnership

NHS Trust

A university teaching and research-active Trust

Families, Young People, Children, Learning
Disabilities, and Autism Services (FYPCLDA)

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Date 24/08/2026

**TEST DATA ONLY - ALL PEOPLE, ADDRESSES, SCHOOLS, SERVICES, IDENTIFIERS AND
DIAGNOSES ARE INVENTED**

For the attention of the SEND service

Re: Maren Quill-Evernight **DOB:** 22 September 2012 **NHS No:** ...

Statutory Education, Health and Care Plan Assessment Contributions from Health

Name: Maren Quill-Evernight	NHS Number:
DOB: 22 September 2012	Sex: Female
Address: 88 Meridian Hollow, New Calder Reach, Leicestershire	
Postcode: LE0 7XY	Home language(s) if other than English:

Contributions Requested

This synthetic health advice has been created solely to test document handling, extraction and EHCP workflows. It describes an intentionally complex, fictional health profile and should not be interpreted as genuine clinical advice, diagnosis, prescribing information or a record belonging to a real person. The health information has been written to align with Maren's synthetic Education Advice while remaining clearly invented.

Medical History

In this wholly invented history, Maren was born following an uncomplicated fictional pregnancy and delivery. Early physical growth remained within the fictional expected range. Feeding variety was narrow from the preschool years because of texture, smell and temperature preferences. Sleep onset was delayed and sleep was easily disturbed by environmental noise. Early vocabulary appeared advanced for preferred topics, while reciprocal interaction, motor sequencing, flexible play and awareness of internal body signals developed less consistently.

At primary-school age, the synthetic record describes increasing fatigue during handwriting, difficulty coordinating unfamiliar movement sequences, discomfort in noisy assemblies, and prolonged recovery following busy days. A fictional sensory and motor participation review at age eight identified reduced postural endurance, variable bilateral coordination and inefficient pencil pressure. A fictional language-processing review at age nine identified difficulty retaining spoken sequences and interpreting implied meaning.

Following transition to secondary education, the combination of multiple classrooms, crowded corridors, fluorescent lighting, competing voices, organisational demands and increased written output was associated with headaches described in the test history, nausea-like sensations during overload, reduced speech, freezing in crowded movement and absence linked to anticipatory distress. These symptoms are invented and have no connection to a real child.

Health Needs**Outcomes**

By the end of Key Stage 4, Maren will recognise and communicate relevant health, sensory and fatigue needs and use agreed adjustments across education, community and health settings with increasing independence.

By the end of Key Stage 4, Maren will manage a personalised routine for hydration, food, movement, rest, appointments and essential health information with no more than one prompt for familiar activities.

Within 12 months, Maren will sustain access to a 30-minute curriculum or community activity using agreed seating, sensory, movement and recording adjustments, with recovery remaining within the individually agreed window in 4 out of 5 opportunities.

Health Provision

Individual health and sensory access plan covering triggers, early indicators, communication, environmental adjustments, recovery and escalation. Health advice coordinator with SENCO, Maren and family

Structured interoception programme using body maps, scheduled checks and concrete action plans for hydration, food, temperature, pain, toileting and fatigue. Occupational participation consultant with trained key adult

Proactive sensory-motor routine selected from an individual menu, including movement, proprioceptive activity and access to quiet recovery. Trained school staff under fictional occupational guidance

Supportive seating, foot support, adjustable work surface and positioning review across teaching areas. Occupational participation consultant and school premises/SEN staff

Alternative recording methods, including laptop, speech-to-text, text-to-speech, prepared notes or scribe; handwriting limited to short purposeful samples. All education staff with assistive-technology lead

Please see attached contributions from:

Questions relating to your child's/ young person statutory health advice must first be discussed with the named professional (as listed above).

Copies to

CC:
Parent/Carer ...
Address ...